

Verified patient data, ready the moment a patient walks in.

A trusted-data layer that gives clinics and labs a single, verified patient record — with the source shown on every value, under patient consent.

Fragmented records cost time and safety.

Every new patient arrives with a history scattered across hospitals, labs and clinics that don't share data. Staff rebuild it by hand, every visit — and clinicians often act before it's complete.

~14 min

Lost per intake

Staff time collecting, re-keying and verifying history that already exists elsewhere.

↑ risk

Acting on guesswork

Allergies missed, medications unknown, blood type unconfirmed at the point of care.

0

Shared source of truth

No common verified record moves between providers, so every visit starts from zero.

One verified record. Source on every value.

XeivoraMed gives clinics and labs a single, verified patient record — blood group, allergies, medications and recent labs — with the source shown on every value. Less time rebuilding history. Fewer decisions made on guesswork.

Green = confirmed from a named source. **Amber** = patient-entered, to double-check.

VERIFIED PATIENT RECORD

Blood group

O Positive

Verified · partner lab

Allergies

Penicillin

Verified · hospital discharge

Medications

Amlodipine 5mg

Verified · prescription

Organ donor

Registered

Self-reported

03 — HOW IT WORKS

A three-way handshake.

Government confirms identity. Certified providers supply the data. The patient grants permission. XeivoraMed connects the three into one verified record.



IDENTITY

Verified by government ID

Confirmed against the national NIC or passport registry — a trusted root no health app can self-report.



DATA

Supplied by certified providers

Labs and clinics in the network contribute verified results directly, so every value carries a real, named source.



PERMISSION

Released by patient consent

The patient grants access once. Every pull is logged, scoped and revocable — consent is the key to the record.

A crowded space — but no one owns verified, portable data.

Health record tools exist. None combine government-verified identity with document-verified medical data that travels with the patient across providers.

	Portable across providers	Government ID verified	Medical data verified (not self-report)
Hospital EMRs (Epic, Cerner)	No — locked to one provider	No	Yes, but siloed
Patient portals / self-report apps	Partially	No	No — self-entered
ID verification tools (Onfido, Truepic)	N/A — identity only	Yes	N/A — no health data
XeivoraMed	Yes	Yes	Yes — document-verified

Built for the people who treat patients.

Clinics and labs are the customer. The patient is the beneficiary — and stays in control.



Clinics

Pull a verified record at intake instead of rebuilding history by hand. Less paperwork, fewer blind spots, faster throughput.



Labs

Make results portable for patients and become a certified source of trusted data in the network.



Patients

One verified health identity that travels with them, with consent and a full record of every access.

How verified data actually moves.

A consent layer and a standards-based (FHIR) connector are the technology. The access right behind them is the real moat — built through three routes.

1

Patient access rights

The patient exercises their legal right to their own records and brings them in. Available today — right for the first pilot.

Pilot now

2

Provider agreements

Direct FHIR integration with a lab or clinic, behind a data-sharing agreement and security review. The real liquidity engine.

Scales the model

3

National exchange

Connect once to a national health-data layer instead of provider-by-provider. The leverage play.

Long-term scale

Anchor with government. Win with clinics. Scale.

Phase 1

Identity anchor

Integrate national ID verification and secure data-protection sign-off.

Months 1–3

Phase 2

Private wedge

Pilot verified onboarding with boutique clinics; prove time saved per patient.

Months 3–6

Phase 3

Data liquidity

Connect a major lab network; prove verified data moving lab → record → doctor.

Months 6–12

Phase 4

National scale

Bring evidence to health systems; become the default trusted-data layer.

Year 2+

UK headquartered. Built for global health systems.

Revenue model

- Customers: clinics & labs (B2B).
- They pay for verified-data access, faster intake and reduced liability.
- Patients use it free — they are the beneficiary, in control of consent.
- Likely pricing: per-clinic subscription or per-verified-record — to be validated in pilots.

Why this structure

- UK entity: access to UK/EU investors, NHS-aligned pathways, and clear regulatory footing from day one.
- The verification model — government ID + document-verified medical data — is designed to be jurisdiction-agnostic, adapting to each market's ID and data-protection framework.
- Founder's cross-border background (UK, medical training abroad) supports early conversations across multiple markets in parallel.
- First provider agreements will determine the first live market — the architecture is not locked to one country.

A solo founder building at the intersection of medicine and product.



Luxshan Nadarasa — Founder

First-year medical student (Belarus) · Previously Biomedical Science, Anglia Ruskin University (UK)

Building XeivoraMed solo — leading product design, verification model, and business strategy, now recruiting a founding developer. Clinical training gives direct, firsthand insight into why unverifiable patient data is a real risk at the point of care, not a hypothetical one.

Also founder, Ed Higher

Honest gap: no technical co-founder or hire yet. Closing this — a developer for the first 6-month build — is the top priority for this raise.

Built, live, and honest about the stage.



Working prototype, live

Public demo at xeivora.com showing the full verified-record flow with demo data.



Provenance engine built

Every value tagged verified vs self-reported, with consent and an access log.



First conversations open

Engaging hospital groups, telecom partners, and a UK health-innovation network for pilot pathways.

Honest scope: pilot stage · demo data · not yet for clinical use · no real patient data until governance & regulatory steps are complete.

Raising \$30,000 to fund a 6-month build-and-pilot phase.

Pre-seed. Use of funds is scoped tightly to one outcome: a working pilot with a signed clinic partner within 6 months.

Use of funds (6 months)

Developer (6 mo)	\$12,000
Founder stipend	\$6,000
Legal & compliance (PDPA/data protection, incorporation)	\$3,000
Pilot & clinic outreach	\$1,500
Identity/document verification APIs	\$1,800
Infrastructure & tooling	\$1,800
Buffer	\$2,600
Total	\$30,000

What this unlocks

- A founding developer, full-time for 6 months
- PDPA-compliant data handling before any real patient data is processed
- A signed pilot with one clinic or lab
- Proof of time-saved-per-patient — the metric that de-risks the next round

What we're looking for.

- **A first pilot** — One clinic or lab, anywhere, to run verified intake and prove time saved.
- **\$30,000 pre-seed** — To fund a founding developer and a focused 6-month build-and-pilot phase.
- **The right introductions** — To health systems, providers and partners globally who can open the first door.